Billing and Coding Education

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PREMIER INPATIENT PARTNERS

Section 1: Overview



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Main CPT wRVU Values

Inpatient Initial Visit	Service	Minutes	wRVU	SHM Bell Curve
99221	H/P	30	1.92	3%
99222	H/P	50	2.61	22%
99223	H/P	70	3.86	75%
Inpatient F/U Visit	**Service**	**Minutes**	**wRVU**	**SHM Bell Curve**
99231	F/U	15	0.76	4%
99232	F/U	25	1.39	63%
99233	F/U	35	2.00	33%
Inpatient D/C Visit	**Service**	**Minutes**	**wRVU**	**SHM Bell Curve**
99238	D/C & Death D/C	<30	1.28	15%
99239	D/C & Death D/C	>30	1.90	85%
Observation Initial Visit	**Service**	**Minutes**	**wRVU**	**SHM Bell Curve**
99218	H/P	30	1.92	3%
99219	H/P	50	2.60	22%
99220	H/P	70	3.56	75%
Observation F/U Visit	**Service**	**Minutes**	**wRVU**	**SHM Bell Curve**
99224	F/U	15	0.76	4%
99225	F/U	25	1.39	63%
99226	F/U	35	2.00	33%
Observation D/C	**Service**	**Minutes**	**wRVU**	
99217	D/C & Death D/C	N/A	1.28	
Same Day Admission /Discharge	**Service**	**Minutes**	**wRVU**	
99234	Same Day Admit / Discharge	40	2.56	
99235	Same Day Admit / Discharge	50	3.24	
99236	Same Day Admit / Discharge	55	4.20	

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Components Of Documentation

- History
  - HPI
  - Past Family, Surgical, Social History
  - ROS
- Physical Exam
- Medical Decision Making

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History

- Chief Complaint
- HPI
  - Location, Severity, Quality, Context, Timing, Duration, Modifying Factors, Associated signs or symptoms
  - State of three chronic problems
    - HTN stable DM2 stable, Dyslipidemia stable
- PFSH
  - Family, Surgical, Social
- ROS

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Physical Exam

- Based on 14 Organ Systems
- Constitutional
  - Three vital signs
  - General Appearance
- Eyes
  - Inspection of conjunctivae or eyelids
  - Examination of pupils and irises (PERRLA)
  - Ophthalmic exam
- ENMT
  - External appearance of ears and nose
  - Assessment of hearing
  - Inspection of nasal mucosa, septum, and turbinates
  - Inspection of lips, teeth, and gums
  - Examination of oropharynx

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Physical Exam

- Neck
  - Exam of neck (masses, symmetry, tracheal position)
  - Examination of thyroid (masses, nodules, tenderness)
- Respiratory
  - Assessment of respiratory effort
  - Palpation of chest (tactile fremitus)
  - Auscultation of lungs
  - Percussion of chest
- Cardiovascular
  - Palpation of heart (PMI)
  - Auscultation of heart
  - Assessment of lower extremity edema
  - Carotids
  - Pedal Pulses

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Physical Exam

- GI
  - Examination of the abdomen (masses or tenderness)
  - Examination of liver and spleen
  - Examination or absence of hernias
  - Examination of anus, perineum, rectum
- Genitourinary
  - Examination of scrotal contents (testicular mass)
  - Digital rectal exam of prostate
  - Examination of penis
  - Examination of bladder (fullness, masses, tenderness)
- Skin
  - Inspection of skin (rashes, ulcers, lesions)
  - Palpation of skin (tenting, turgor)

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Physical Exam

- MSK
  - Examination of gait and station
  - Inspection of digits and nails (cyanosis, clubbing, ischemia)
  - Examination of joints, muscles
    - Range of motion
    - Symmetry
    - Tenderness of effusions
    - Dislocation
    - Muscle strength and tone
- Neurological
  - CN with any deficits
  - Examination of DTRs
  - Examination of sensation

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Physical Exam

- Neurological
  - CN with any deficits
  - Examination of DTRs
  - Examination of sensation
- Psychiatric
  - Judgement and insight
  - Mental status
    - Orientation
    - Mood and affect
    - Recent and remote memory

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Review of Symtoms

- Const
  - No Lack of energy, unexplained weight gain or weight loss, loss of appetite, fever, night sweats
- Ears, Nose, Mouth & Throat
  - No Difficulty with hearing, sinus problems, runny nose, post-nasal drip, ringing in ears, mouth sores, loose teeth, ear pain, nosebleeds, sore throat
- C-V (Heart & Blood Vessels)
  - No chest pains swelling of feet or legs, pain in legs with walking.

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Review of Symtoms

- Resp. (Lungs & Breathing)
  - No Shortness of breath, night sweats, wheezing, sputum production, pleurisy, coughing up blood
- GI (Stomach & Intestines)
  - No Heartburn, constipation, diarrhea, abdominal pain, difficulty swallowing, nausea, vomiting, blood in stools
- GU (Kidney & Bladder)
  - No Painful urination, frequent urination, urgency, prostate problems, bladder problems, impotence.
- MS (Muscles, Bones, Joints)
  - No Joint pain, aching muscles, shoulder pain, swelling of joints, joint deformities, back pain

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Review of Symtoms

- Neurologic (Brain & Nerves)
  - No Frequent headaches, double vision, weakness, change in sensation, problems with walking or balance, dizziness, tremor, loss of consciousness, uncontrolled motions, episodes of visual loss.
- Psychiatric (Mood & Thinking)
  - No Insomnia, irritability, depression, anxiety, recurrent bad thoughts, mood swings, hallucinations, compulsions.
- Endocrinologic (Glands)
  - No Intolerance to heat or cold, menstrual irregularities, frequent hunger/urination/thirst, changes in sex drive.
- Hematologic (Blood/Lymph)
  - No Easy bleeding, easy bruising

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Medical Decision Making

Based on three categories

Only need 2 out of 3 to meet for MDM

- Data
- Problem Points
- Level of Risk
  - Presenting Problem, Diagnostic Procedures Ordered, and Management Options
  - Only need 1/3 components to determine level of risk
  - Always choose the highest level of risk

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Problem Points

- Self limited or minor (max 2) - 1 point
- Established problem, stable - 1 point
- Established problem, worsening - 2 points
- New problem, no workup needed - 1 problem max ex/ dyslipidemia, started on statin - 3 points
- New problem, additional workup - ex/ acute renal failure, renal ultrasound - 4 points

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Points of Data Reviewed

- Review/order clinical lab test - 1 point
- Review/order X rays - 1 point
- Review/order echo, EKG, PFTs - 1 point
- Discussing test results with consultant - 1 point
- Independent review of CT, CXR, EKG - 2 points
  - I personally reviewed…
- Decision to obtain old records - 1 point
- Review and summarizing records - 2 points

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Points of Data Reviewed

Level of Risk	Presenting Problem	Diagnostic Procedures Ordered	Management Options
Minimal	• One self-limited or minor problem, e.g., cold, insect bite, tinea corporis	Laboratory tests requiring venipuncture • Chest x-rays • EKG/EEG • Urinalysis • Ultrasound, e.g., echo • KOH prep	• Rest • Gargles • Elastic bandages • Superficial dressings
Low	• Two or more self-limited or minor problems • One stable chronic illness, e.g., well controlled hypertension or non-insulin dependent diabetes, cataract, BPH • Acute uncomplicated illness or injury, e.g., cystitis, allergic rhinitis, simple sprain	• Physiologic tests not under stress, e.g.,pulmonary function tests • Non-cardiovascular imaging studies with contrast, e.g., barium enema • Superficial needle biopsies • Clincal laboratory tests requiring arterial puncture • Skin biopsies	• Over-the-counter drugs • Minor surgery with no identified risk factors • Physical therapy • Occupational therapy • IV fluids without additives

Points of Data Reviewed

Level of Risk	Presenting Problem	Diagnostic Procedures Ordered	Management Options
Moderate	• One or more chronic illnesses with mild exacerbation, progression, or side effects of treatment • Two or more stable chronic illnesses • Undiagnosed new problem with uncertain prognosis, e.g., lump in breast • Acute illness with systemic symptoms, e.g., pyelonephritis, pneumonitis, colitis • Acute complicated injury, e.g., head injury with brief loss of consciousness	• Physiologic tests under stress, e.g., cardiac stress test, fetal contraction stress test • Diagnostic endoscopies with no identified risk factors • Deep needle or incisional biopsy • Cardiovascular imaging studies with contrast and no identified risk factors, e.g., arteriogram cardiac cath • Obtain fluid from body cavity, e.g., lumbar puncture, thoracentesis, culdocentesis	• Minor surgery with identified risk factors • Elective major surgery (open, percutaneous or endoscopic) with no identified risk factors • Prescription drug management • Therapeutic nuclear medicine • IV fluids with addititives • Closed treatment of fracture or dislocation without manipulation

Points of Data Reviewed

Level of Risk	Presenting Problem	Diagnostic Procedures Ordered	Management Options
High	• One or more chronic illnesses with severe exacerbation, progression, or side effects of treatment • Acute or chronic illnesses or injuries that may pose a threat to life or bodily function, e.g., multiple trauma, acute MI, pulmonary embolus, severe respiratory distress, progressive severe rheumatoid arthritis, psychiatric illness with potential threat to self or others, peritonitis, acute renal failure • An abrupt change in neurologic status, e.g., seizure, TIA, weakness or sensory loss	• Cardiovascular imaging studies with contrast with identified risk factors • Cardiac electrophysiological tests • Diagnostic endoscopies with identified risk factors • Discography	• Elective major surgery (open, percutaneous or endoscopic with identified risk factors) • Emergency major surgery (open, percutaneous or endoscopic) • Parenteral controlled substances (IV pain meds) • Drug therapy requiring intensive monitoring for toxicity • Decision not to resuscitate or to de-escalate care because of poor prognosis

Drugs Considered High Risk

Drug Category	Drugs in that Category	Treatment Use
Cardiac drugs	Digoxin, digitoxin, quinidine, procainamide, amiodarone	Congestive heart failure, angina, arrhythmias
Antibiotics	Aminoglycosides (gentamicin, tobramycin, amikacin) Vancomycin, Chloramphenicol	Infections with bacteria that are resistant to less toxic antibiotics
Antiepileptics	Phenobarbital, phenytoin, valproic acid, carbamazepine, ethosuximide, sometimes gabapentin, lamotrigine	Epilepsy, prevention of seizures, sometimes to stabilize moods
Bronchodilators	Theophylline, caffeine	Asthma, Chronic obstructive pulmonary disorder (COPD), neonatal apnea
Immunosuppressants	Cyclosporine, tacrolimus, sirolimus, mycophenolate mofetil, azathioprine	Prevent rejection of transplanted organs, autoimmune disorders
Anti-cancer drugs	All cytotoxic agents	Multiple malignancies
Psychiatric drugs	Lithium, valproic acid, some antidepressants (imipramine, amitriptyline, nortriptyline, doxepin, desipramine)	Bipolar disorder (manic depression), depression
Protease inhibitors	Indinavir, ritonavir, lopinavir, saquinavir, atazanavir, nelfinavir	HIV/AIDS

Medical Decision Making

	Problem Points	Data Points	Risk
Straightforward	≤ 1	≤ 1	Minimal risk
Low	2	2	Low
Moderate	3	3	Moderate
High	≥ 4	≥ 4	High

For risk, you only need one component to determine level of risk

Need 2 out of 3 components to determine medical decision making

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Section 1
Quiz



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1) What are the key components to medical documentation
   a. History
   b. Physical Exam
   c. Medical Decision Making
   d. All of the above

2) How many components of the Past Medical, Family, and Social History are required for a Level 3 H&P
   a. 1
   b. 2
   c. 3
   d. 4

3) What is needed for a level 3 HPI
   a. 1-3 HPI elements
   b. 1-2 comorbidities
   c. Four or more HPI elements are needed or the status of three chronic or inactive problems
   d. Six HPI elements

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Section 2: Admission / History & Physical

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Admission / H&P

- Work backwards with medical decision making
- MDM is split up into Problem points, data points, and risk
- After establishing MDM, look at HPI and Physical Exam
- For Admission H&Ps, you'll need 3/3 key components for billing

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Level 2 H&P

- History
  - Comprehensive
    - Extended HPI or status of three chronic issues, 3/3 PFSH, and 10 ROS
- Physical Exam
  - Comprehensive
    - 2 Bullets from Nine Organ Systems
- MDM
  - Moderate (NEED ONLY 2/3 COLUMNS TO QUALIFY)

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Level 3 H&P

- History
  - Comprehensive
    - Extended HPI or status of three chronic issues, 3/3 PFSH, and 10 ROS
- Physical Exam
  - Comprehensive
    - 2 bullets from NINE organ systems
- MDM
  - High (NEED 2/3 COLUMS TO QUALIFY)

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Physical Exam for H&P

- NAD
  - Vitals (3 bullets)
- Eyes
  - Pink conjunctivae, no ptosis, PERRL
- Neck
  - No masses, trachea midline, no thyromegaly
- Lungs
  - Clear to auscultation with normal respiratory effort
- CVS
  - RRR, no murmurs, rubs gallops, normal PMI, no edema, no carotid bruits

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Physical Exam for H&P

- GI
  - Soft, NT, ND, no hepatosplenomegaly
- MSK
  - Normal gait, no clubbing, no cyanosis, FROM in all extremities
- Skin
  - Warm and dry, no rashes, no ulcers, no lesions. Normal texture and turgor
- Psych
  - AAOX3 with appropriate affect with intact judgement and insight

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Section 2
Quiz



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1) What is needed for a level 3 H&P physical exam
   a. 2-4 bullets
   b. 9 bullets
   c. 12 bullets
   d. 2 bullets from EACH of NINE organ systems

2) How many qualifying key components are needed for all H&Ps
   a. 1/3 key components
   b. 2/3 key components
   c. 3/3 key components
   d. None of the above

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Section 3:
Progress Notes / Follow Up

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Progress Notes

- Need only to fulfill 2 out of 3 key elements of History, Physical, and MDM
- Work backwards again with medical decision making

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Level 2 Progress Note

- History
  - Expanded Problem Focused History
    - Brief HPI (1-3 HPI elements)
  - No PFSH, no need for PFSH on progress notes
  - 1 ROS
- Exam
  - Expanded Problem Focused Exam
    - 6-11 Bullets
- Medical Decision Making
  - Moderate

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Level 3 H&P

- History
  - Comprehensive
    - Extended HPI or status of three chronic issues, 3/3 PFSH, and 10 ROS
- Physical Exam
  - Comprehensive
    - 2 bullets from NINE organ systems
- MDM
  - High (NEED 2/3 COLUMS TO QUALIFY)

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Surgical Risk Factors

1- Very Low Risk	**Procedures that usually require only minimal or moderate sedation and have few physiologic effects** •Eye surgery •GI endoscopy (without stents) •Dental procedures
2- Low Risk	**Procedures associated with minimal physiologic effect** •Hernia repair •ENT procedures without planned flap or neck dissection •Diagnostic cardiac catheterization •Interventional radiology •GI endoscopy with stent placement •Cystoscopy
3- Intermediate Risk	**Procedures associated with moderate changes in hemodynamics, risk of blood loss** •Intracranial and spine surgery •Gynecologic and urologic surgery •Intra-abdominal surgery without bowel resection •Intra-thoracic surgery without lung resection •Cardiac catheterization procedures including electrophysiology studies, ablations, AICD, pacemaker
4- High Risk	**Procedures with possible significant effect on hemodynamics, blood loss** •Colorectal surgery with bowel resection •Kidney transplant •Major joint replacement (shoulder, knee, and hip) •Open radical prostatectomy, cystectomy •Major oncologic general surgery or gynecologic surgery •Major oncologic head and neck surgery
5- Very High Risk	**Procedures with major impact on hemodynamics, fluid shifts, possible major blood loss** •Aortic surgery •Cardiac surgery •Intra-thoracic procedures with lung resection •Major transplant surgery (heart, lung, liver)

Physical Exam for L3 H&P

- NAD, Vitals (3 bullets)
- Eyes:Pink conjunctivae, no ptosis, PERRL
- Neck: No masses, trachea midline, no thyromegaly
- Lungs: Clear to auscultation with normal respiratory effort
- CVS: RRR, no murmurs, rubs gallops, normal PMI, no edema, no carotid bruits
- GI Soft, NT, ND, no hepatosplenomegaly
- MSK: Normal gait, no clubbing, no cyanosis, FROM in all extremities
- Skin: Warm and dry, no rashes, no ulcers, no lesions. Normal texture and turgor
- Psych AAOX3 with appropriate affect with intact judgement and insight

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Progress Notes

- Need only to fulfill 2 out of 3 key elements of History, Physical, and MDM
- Work backwards again with medical decision making

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Level 2 Progress Note

- History
  - Expanded Problem Focused History
    - Brief HPI (1-3 HPI elements)
  - No PFSH, no need for PFSH on progress notes
  - 1 ROS
- Exam
  - Expanded Problem Focused Exam
    - 6-11 Bullets
- Medical Decision Making
  - Moderate

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Level 3 Progress Note

- History
  - Detailed History
    - Extended HPI (4 HPI elements or status of three chronic illnesses)
  - 1/3 PFSH, but for progress notes not needed
  - 2-9 ROS
- Physical Exam
  - Detailed
    - 12 Bullets
- Medical Decision Making
  - High (NEED 2/3 components)

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Physical Exam Level 3 Progress Note

- Gen: NAD, 3 vital signs
- Neck trachea is midline, no JVD
- Lungs CTA B/L, no crackles, no wheezes, normal respiratory effort
- Heart RRR, no MRGs, normal PMI, no pitting edema
- Abd soft, NT/ND, +BS, no hepatosplenomegaly
- Skin warm and dry, well perfused
- Psych AAOX3

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MDM – Example 1

- You are seeing a patient with improving COPD exacerbation
- Has history of HTN and CAD which are stable
- Reviewed labs and are WNL
- Switch patient from IV Steroids to oral Steroids
- Order AM Labs for morning
- What Level of MDM is this?

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MDM – Example 2

- Called by ER to see a 55 y/o male with SOB
- Patient with history of CAD and CHF
- Personally reviewed EKG tracing
- Reviewed dictated CXR
- Reviewed labs
- Admit patient and ordered IV diuretics
- Ordered cardiac enzymes for CAD
- Ordered ECHO
- What level of MDM is this?

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MDM – Example 3

- You are called to see a 30 y/o patient with asthma with shortness of breath
- Labs reviewed
- Personally reviewed CXR
- Patient with asthma exacerbation, needs IV Steroids and Nebulizers and ordered AM labs
- What is the MDM?

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MDM – Example 4

- You are called to evaluate a 80 y/o female nursing home resident with confusion
- CT Head negative
- Labs reviewed and reveal hypernatremia and acute renal failure
- CXR personally reviewed and shows right middle lobe pneumonia
- Order blood cultures, IV abx, and IVF
- What is MDM?

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MDM – Example 5

- You are following up on a patient with worsening CHF and HTN and uncontrolled DM and you see that the patient has developed acute renal failure
- Patient still complains of shortness of breath
- Labs reviewed and show hyponatremia and acute renal failure
- Personally reviewed CXR
- Adjusted IV diuretics
- Broadened insulin sliding scale coverage
- Ordered renal US
- What level of MDM is this?

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Section 3
Quiz



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1) When completing a level 3 progress note, how many components of the past medical, family, and social history are required?

a. 1
b. 2
c. 3
d. None

2) For a level 3 progress note, how many systems should be reviewed

a. 1
b. 2
c. 5
d. 7
e. 10

3) How many qualifying key components are needed for hospital progress notes?

a. 2/3
b. 3/3
c. 1/3
d. None are required

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4) For level 3 progress notes, how much time must you document

a. 15 minutes
b. 25 minutes
c. 35 minutes

5) All of these are considered high risk procedures except

a. Emergency major surgery
b. Cardiac catheterization
c. Nephrectomy
d. All of the above

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Section 4: Discharges

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Discharge Notes

- Time Based coding
- To get credit for discharges over the 30 minutes, must document time.
- If discharge is less than 30 minutes, it is not required to document time.
- Discharge time includes
  - Non face to face time spent on the floor
  - Coordinating discharge with patient
  - Patient's family
  - Case coordinators
  - Documenting the discharge summary

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Discharge Notes

- For discharge summaries, you must document:
  - Principal Diagnoses
  - Physical Exam
  - Description of Hospitalization
  - Disposition
  - Follow up Care
  - Discharge Medications

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Discharge Notes

- For discharge summaries, you must document:
  - Principal Diagnoses
  - Physical Exam
  - Description of Hospitalization
  - Disposition
  - Follow up Care
  - Discharge Medications

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Section 5: Other Coding Considerations



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Other CPT & wRVU Values

Consults Inpatient / Observation	Service	Minutes	wRVU	
99251	Consult	15	1.00	
99252	Consult	30	1.50	
99253	Consult	40	2.27	
99254	Consult	60	3.29	
99255	Consult	80	4.00	
Critical Care	**Service**	**Minutes**	**wRVU**	
99291	Critical Care	30-74	4.50	
99292	Critical Care - each additional	30	2.25	
Prolonged Service	**Service**	**Minutes**	**wRVU**	
99356	Prolonged Inpatient	60	1.71	
99357	Prolonged Inpatient - each additional	30	1.71	
Miscellaneous	**Service**	**Minutes**	**wRVU**	**SHM Bell Curve**
99406	Tobacco Cessation	3-10	0.24	
99407	Tobacco Cessation	10	0.50	
99408	Alcohol / Substance Screen	15-30	0.65	
99409	Alcohol / Substance Screen	30	1.30	
99497	ACP	30	1.50	78%
99498	ACP - each additional	30	1.40	22%
S-001	Not Seen	N/A	N/A	

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Advanced Care Planning – First 30 Minutes

- Time Based Coding
  - Requires at least 16 minutes
- Code Status, Healthcare Proxy, and Advanced Directives (Living Will)
- If patient's status is worsening, have a "goals of care" conversation and Consider a Palliative or Hospice Consult
- 16 minutes total time spent in Advance Care Planning and associated care coordination separate from other services provided

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Critical Care

- Critical care is spent based on time
- Need to document more than 30 minutes of critical care time was spent excluding time spent on procedures
- Called at bedside to evaluate opiate overdose from PCA pump with shallow breathing
- Called to bedside for chest pain
- Called to bedside for patient with acute stroke symptoms
- New onset seizure on floor patient with multiple problems
- Evaluate on floor for hypotension

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Critical Care

- Admission: CHF with IV Lasix and Bipap
- Admission or on floors: SVT with Adenosine
- Admission: Acute renal failure, hyperkalemia with need for acute HD
- Admission: Symptomatic hyponatremia
- Admission: PNA, Asthma/COPD with Bipap
- Admission: GI bleed with SIRS criteria
- Admission: DKA with insulin drip
- Admission: HTN emergency/urgency requiring IV therapy
- Admission: Afib with RVR requiring Cardizem drip

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Coding Based on Time

- Spent a total of 35 minutes face to face with patient on the floor and over half of that time was devoted to counseling and coordinating care in regards to…
- This is for Progress note Level 3 documentation

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Prolonged Services Time

- A service is only prolonged when it spans a significant amount of time beyond the typical time allotted for that E/M encounter
  - Level 2 progress note, E/M time is 25 minutes
  - Level 3 progress note, E/M time is 35 minutes
- The clock starts after you used up the "normal" time allotted for an E/M encounter
- Must document the nature of the prolonged service
- Must document time spent, start time and stop time
- In order to bill for prolonged services time, you must spend at least 30 minutes above and beyond the usual time for any given encounter. Threshold for the first hour of prolonged services is 30 minutes
- Threshold for each additional 30 minutes beyond the first hour is 15 minutes

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Examples of Prolonged Services

- In the morning, you perform a level 2 progress note (99232) on patient with CKD approaching ESRD.
- Encounter takes about 20 minutes
- Later that day, you spend an additional 36 minutes with the patient and family discussing the risks and benefits of various renal replacement therapies
- Time Spent on initial encounter was 20 minutes (8:15 AM to 8:35 AM)
- Later, you add an addendum for the 36 minutes spent
- "I came back to see the patient for a second visit in the presence of his family to discuss various dialysis options. I spent 36 minutes face to face with the patient (2:10 PM to 2:46 PM).
- Total time spent: 56 minutes

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Non Face-to-Face Prolonged Services Time

- Similar timing for face-to-face encounter
- Used to report time spent not face-to-face with the patient, but exclusively devoted to that patients care
- Document time spent
- Document the medical necessity and nature of the prolonged services. Does not have to be continuous.
- Example: "A 65 year-old new patient with multiple problems was seen and evaluated. After the visit, I spent a significant amount of time talking with the patient's daughter, reviewing complex, detailed medical records transferred from previous physicians, and completing a comprehensive treatment plan."

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Smoking Cessation Counseling

- Greater than 3 minutes required in order to bill
- Documentation should include the amount of time spent counseling the patient, recommended treatment plan and the patient's response to counseling.
- Best practice statement example: "Patient was counseled on smoking cessation for 6 minutes, RX for nicotine patch offered, patient refuses and states does not want to quit".

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Section 5
Quiz



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1) In order to bill for critical care, how much time must be spent
   a. 15 minutes
   b. 16 minutes
   c. 25 minutes
   d. 30 minutes

2) Critical Care services include all except:
   a. Patient in CHF requiring Bipap
   b. Afib with RVR requiring Adenosine or Cardizem drip
   c. Admitted patient, progress note written, and now complaining of new onset chest pain
   d. Patient requiring intubation
   e. All of the above

3) Which is true of prolonged services
   a. Can be submitted with other E/M codes
   b. You must record start and stop times
   c. Time spent does NOT have to be continuous
   d. All of the above

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4) How much time must be spent before you can bill for prolonged services
   a. 5 minutes
   b. 15 minutes
   c. 20 minutes
   d. 30 minutes

5) For advanced care planning, what should be documented?
   a. Does patient have a living will
   b. Does patient have a POA or Health Care Proxy
   c. Minimum 16 minutes discussing advance directives and end of life planning with the patient and patient's family required in order to bill.
   d. All of the above

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Miscellaneous Quiz



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1) What level of risk is associated with IV Pain medications
   a. Minimal
   b. Low
   c. Moderate
   d. High

2) How many data points do you get for personally reviewing an EKG and a CXR
   a. 1
   b. 2
   c. 3
   d. 4

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3) How many data points do you get for reviewing and ordering labs
   a. 1
   b. 2
   c. 3
   d. 4

4) How many data points do you receive for stating a new problem but no further workup is required
   a. 1
   b. 2
   c. 3
   d. 4

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